Analysis of the clinic's operating metrics over 15 months
to forecast results and recommend a strategy that maximises profit in quarter 5.

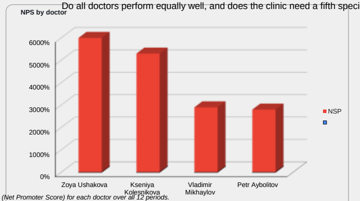
Clients acquired
3238
of 4,228 residents (77% know the clinic)

Profit in Period 12
€3.382
first profitable month

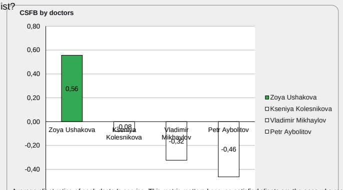
NPS Q1-Q4
Client loyalty increased
from -18% to 48%

Accumulated loss at end of Q4
-€70.558,00

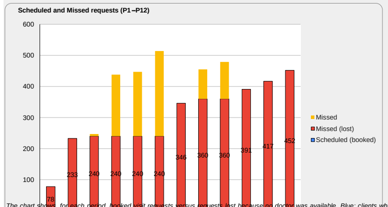
Hypothesis 1: Service quality and doctor workload.
Do all doctors perform equally well, and does the clinic need a fifth specialist?



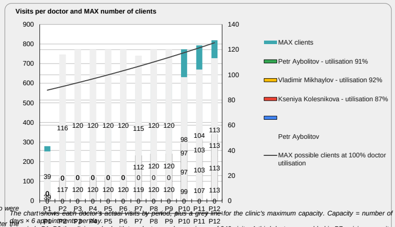
NPS (Net Promoter Score) for each doctor over all 12 periods.
Calculated as the difference between satisfied and dissatisfied clients divided by the total number of visits. Zoya Ushakova leads with an NPS of 60%. Petr Aybolitov and Vladimir Mikhaylov are in the risk zone at 28–29%.
The gap between the best and the worst doctor is more than twofold. Dissatisfied clients do not come back and do not recommend the clinic, which is a direct loss for the referral channel.



Average client rating of each doctor's service. This metric matters because satisfied clients are the ones who recommend the clinic to people they know. The data shows that advertising stopped in period 7 and only the referral program is running.
Only 1 doctor is positive at +0.56; all other doctors are negative.
Petr Aybolitov at −0.46 with 1,305 visits means thousands of clients left with a negative impression of the service. This is what holds back the growth of the referral base.



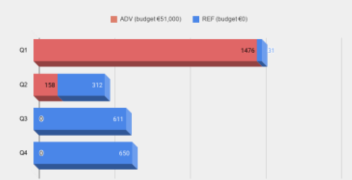
The chart shows, for each period, booked visit requests versus requests lost because no doctor was available. Blue: clients who were booked. Red: clients who wanted to book but could not.
In periods P4–P6 the clinic lost 198 to 274 clients a month. After the third doctor was hired in P7, losses dropped to zero. After the fourth was hired in P10 they stayed at zero and have not grown since.
This directly confirms that the current team of four doctors fully covers demand.



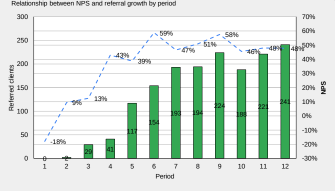
The chart shows each doctor's actual visits by period, plus a grey line for the clinic's maximum capacity. Capacity = number of doctors × 20 days × 6 appointments per day.
In periods P1–P6 the clinic worked with two doctors and a maximum of 240 visits. A third doctor was added in P7, raising capacity to 360.
A fourth doctor was added in P10, capacity 480. Doctor utilisation stays at a steady 87–92%, which is optimal. A fifth doctor is not needed.

Conclusion: Hypothesis 1
Hypothesis confirmed. Doctors' work quality differs considerably: the NPS gap is more than twofold. At the same time clinic utilisation is optimal and no clients are lost. Two recommendations: run customer-service training for Aybolitov and Mikhaylov, and do not hire a fifth doctor at this stage, as it would only add costs without adding revenue.

Hypothesis 2: Profit growth without new clients
How can the clinic grow revenue without adding staff or marketing? How can the referral program be improved?



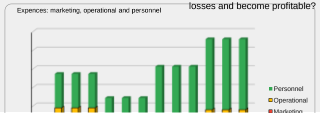
New clients per quarter, stacked by channel: paid advertising (ADV) and referral program (REF).
It shows clearly how the channel mix changed over the year.
In Q1 almost all acquisition came from advertising: 1,476 clients for €45,000. By Q2 referrals were already catching up.
In Q3 and Q4 advertising was switched off completely, yet the clinic gained 611 and 650 new clients purely through referrals, at no cost. The inflow did not drop; it kept growing.

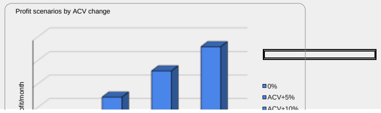


Combo chart: bars show the number of promoters (clients with PS ≥ 9) in each period; the line shows NPS.
There is a clear correlation: when NPS rises, the number of referrals rises in the next period.
This proves a direct link: service quality creates promoters, and promoters bring new clients for free. That is why investing in doctors' service quality pays off better than spending on advertising.

Conclusion: Hypothesis 2
Hypothesis confirmed. The referral channel now matches advertising in effectiveness at zero cost. Clients acquired are split almost evenly: 50.5% advertising vs 49.5% referrals, but the cost is fundamentally different. After advertising was switched off in Q3, the inflow of clients did not decrease. The referral program is fully self-sustaining at this stage.
What can work: maintain and grow the referral program by raising NPS. Every promoter with a score of 9 or higher brings in new clients for free. This is why improving doctors' service quality (Hypothesis 1) directly drives client base growth (Hypothesis 2).

Hypothesis 3: Q5 profit forecast
What increase in the average check would raise the clinic's profitability? How long will it take the clinic to cover its losses and become profitable?





Period		Profit		Total	Q5 parameters	
P1		-€33.434		-€33.434	ACV Q5 (+10%	$259
P2		-€7.909		-€41.343	Q5 costs (bas	107892
P3		-€8.142		-€49.485	Q5 visits	466
P4		-€4.673		-€54.158	Q5 revenue	14595
P5		-€5.392		-€59.550		
P6		€187		-€59.363		
P7		-€3.410		-€62.773		
P8		-€2.097		-€64.870		
P9		-€5.467		-€70.337		
P10		-€1.846		-€72.183		
P11		-€1.757		-€73.940		
P12		€3.382		-€70.558		
P13		€17.788		-€52.770		
P14		€17.788		-€34.983		
P15		€17.788		-€17.195		

Doctors		NSP	Utilisation	Visits	Revenue per d	MAX clients at 10	Months worked
Zoya Ushakova		60	91%	1314	292371	1440	12
Kseniya Kolesnikova		53	87%	313	73741	360	3
Vladimir Mikhaylov		29	92%	665	156738	720	6
Petr Aybolitov		28	91%	1305	298866	1440	12

Doctors	CSFB
Zoya Ushakova	0,56
Kseniya Kolesnikova	-0,08
Vladimir Mikhaylov	-0,32
Petr Aybolitov	-0,46

Doctors		Visits	MAX clients at 100% utilisation
Zoya Ushakova		1314	1440
Kseniya Kolesnikova		313	360
Vladimir Mikhaylov		313	720
Petr Aybolitov		1305	1440

ADV vs REF			
Quarter	ADV		REF
Q1	1476		31
Q2	158		312
Q3	0		611
Q4	0		650
Q5			

Period		Zoya Ushakova	Kseniya Kolesnikova	Vladimir Mikha	Petr Aybolitov	MAX clients	Total
P1		39	0	0	39	240	78
P2		117	0	0	116	240	233
P3		120	0	0	120	240	240
P4		120	0	0	120	240	240
P5		120	0	0	120	240	240
P6		120	0	0	120	240	240
P7		119	0	112	115	360	346
P8		120	0	120	120	360	360
P9		120	0	120	120	360	360
P10		99	97	97	98	480	391
P11		107	103	103	104	480	417
P12		113	113	113	113	480	452
Total		1314	313	665	1305		
MAX clients at 100% utilisation		1440	360	720	1440	3960	

Period		Scheduled	Missed
P1		78	0
P2		233	0
P3		240	7
P4		240	198
P5		240	207
P6		240	274
P7		346	0
P8		360	95
P9		360	119
P10		391	0
P11		417	0
P12		452	0

Quarter	ADV	REF	ADV budget	Budget per client
Q1	1476	31	€45.000	€30
Q2	158	312	€6.000	€38
Q3	0	611	€0	0
Q4	0	650	€0	0
			€51.000	

Relationship between NPS and referral growth by period			
Period	NPS	Referrals next	Promoters
P1	-18%	0	2
P2	9%	2	47
P3	13%	29	52
P4	43%	41	116
P5	39%	117	110
P6	59%	154	153
P7	47%	193	195
P8	51%	194	220
P9	58%	224	234
P10	46%	188	221
P11	48%	221	249
P12	48%	241	264

Period		P1	P2	P3	P4	P5	P6	P7	P8	P9	P10	P11	P12
Marketing		€15.000	€15.000	€15.000	€2.000	€2.000	€2.000	€0	€0	€0	€0	€0	€0
Operational		€14.000	€14.000	€14.000	€14.000	€14.000	€14.000	€21.000	€21.000	€21.000	€28.000	€28.000	€28.000
Personnel		€18.300	€18.300	€18.300	€18.300	€18.300	€18.300	€30.200	€30.200	€30.200	€37.900	€37.900	€37.900

Scenario	Profit/month	ACV	Revenue/month	Costs		Number of visits	Forecast number of visits
0%	€6.694	€235	€109.620,57	€102.927		480	466
ACV+5%	€12.195	€247	€115.122	€102.927		480	466
ACV+10%	€17.788	€259	120714,72	€102.927		480	466
ACV+15%	€22.915	€270	€125.842	€102.927		480	466

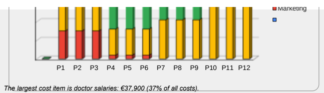
Profit adjusted for forecast future visits

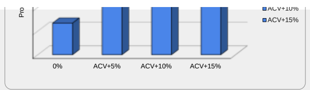
Scenario	Q5 forecast by utilisation, %	Visits	Fixed costs	Service costs	Total costs	Income	Profit per mont	Profit Q5
Utilisation at minim	81,5	391	€65.900	35233	101133	101158	25	75
Utilisation at Q4	87,5	420	€65.900	37846	103746	108661	4915	14745
Utilisation at P12	94,2	452	€65.900	40730	106630	116940	10310	30930
Full utilisation	100	480	€65.900	43253	109153	124184	15031	45093
Average including client growth in Q4	97,1	466			107891,5		14595,101	43785,303

ACV Q5 10%	259
Service cost ratio	0,35
Fixed costs	€65.900

Standard deviation 30,61045573

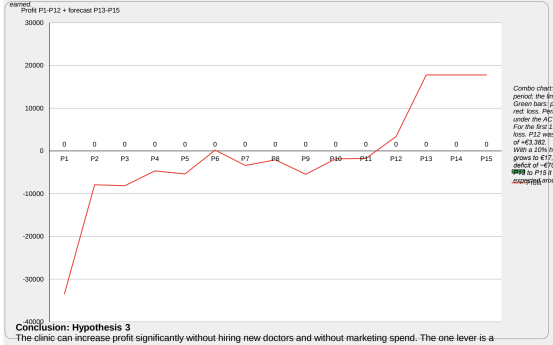
0





The largest cost item is doctor salaries: €37,900 (37% of all costs).
This is a fixed cost that does not depend on the number of clients seen: the clinic pays the same at 391 visits as at 480. That is why utilisation is so critical: more clients at the same salaries means higher profit.
Operating expenses are also fixed: €28,000 (27%).
The only variable cost item is service costs: €34,426 (34%).
The more visits, the higher these costs. The ratio is 34.8% of revenue, so the clinic spends about 35 cents on materials for every euro earned.

We compare four scenarios for the average check: current level, +5%, +10% and +15%.
Y axis: expected monthly profit at 466 visits. The figures account for client growth over the last quarter.
Even with no changes, the clinic at full capacity earns about €6,694 per month. +5% adds another €5,498. +10% gives €17,788 per month. +15% gives the highest profit but risks losing price-sensitive clients. That is why I recommend the +10% scenario: it gives the best balance between profit growth and churn risk.



Combo chart: bars show profit or loss for each period; the line shows the cumulative total.
Green bars: profit; light green: expected profit; red: loss. Periods P13–P15 are the forecast under the ACV +10% strategy.
For the first 11 periods the clinic operated at a loss. P12 was the first period with a solid profit of +€3,382.
With a 10% higher average check in Q5, profit grows to €17,778 per month. The accumulated deficit of −€70,558 starts shrinking fast: from P13 to P15 it falls by €53,363. Full payback is expected around period 16–17.

Conclusion: Hypothesis 3

The clinic can increase profit significantly without hiring new doctors and without marketing spend. The one lever is a 10% increase in the average check. With the current client flow and a stable 90–94% utilisation, this is realistic and requires no additional investment.

Recommended Q5 strategy in three points:

Raise ACV by 10%, positioning it as a premium service rather than a simple price increase. The argument for clients is high-quality treatment and attentive service. Staff should also receive service-quality training.

This alone brings the clinic to a monthly profit of about €17,000 and fully pays off the accumulated deficit by around period 16–17.